OPRAZOL CAPSULES 20MG

Description: Dark peach opaque/Light peach opaque capsules.

Content:
Each capsule contains omeprazole pellets (enteric-coated) eq. to omeprazole 20mg.

Indications:
It is used in the treatment of reflux oesophagitis, duodenal and benign gastric ulcers, and the Zollinger-Ellison syndome.

Pharmacology:
Omeprazole, a member of a new class of antisecretory compounds termed the substituted benzimidazoles. It inhibits secretion of gastric acid and is considered to do so by blocking the enzyme system of H^+/ K^+ ATPase (proton pump) of the gastric parietal cell. Both basal and stimulated acid secretions are inhibited. It is used in the management of gastrointestinal disorders thought to be related to an excess of stomach acid. Omeprazole is acid-labile. The product contains an enteric-coated granule formulation and appears to be dose-dependant, as increasing the dosage has been reported disproportionately to increase plasma concentrations. Omeprazole is almost completely metabolised in the liver. The drug is highly bound (95%) to plasma proteins. Inactive metabolites is rapidly eliminated, mostly in the urine. Although, the elimination half-life from plasma is short, being reported to be about 0.5 - 3 hours, its duration of action with regards to inhibition of acid secretion is much longer allowing it to be used in single daily doses and it is suggested that its distribution to the tissues, and particularly to the gastric parietal cells, accounts for this action.
During treatment with antisecretory medicinal products, serum gastrin increases in response to the decreased acid secretion. Also CgA increases due to decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.
Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

Adverse effects:
Gastrointestinal disorders: Microscopic colitis: Frequency 'not known'. Subacute Cutaneous Lupus Erythematosus (SCLE): Skin and subcutaneous tissue disorders. Frequency 'not known': Subacute cutaneous lupus erythematosus. Interstitial Nephritis: Renal and urinary disorders: Interstitial nephritis. Hypomagnesaemia: Metabolism and nutritional disorders. Frequency "not known": hypomagnesaemia. Fracture: Musculoskeletal disorders. Frequency "uncommon": Fracture of the hip, wrist or spine. Clostridium Difficile Diarrhea: Infections & infestations: Clostridium difficile associated diarrhea. Fundic Gland Polyps (Benign): Gastrointestinal disorders. Frequency "common": Fundic gland polyps (benign). Vitamin B12 Deficiency: Metabolic/Nutritional: Vitamin B12 deficiency

Warnings and Precautions:
Regular Surveillance: Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance. Subacute Cutaneous Lupus Erythematosus (SCLE): Proton pump inhibitors are associated with very infrequent cases of subacute cutaneous lupus erythematosus (SCLE). If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping Oprazol. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors. Hypomagnesaemia: Severe hypomagnesaemia has been reported in patients treated with PPI like Oprazol for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI. For patients expected to be on prolonged treatment or who take PPI with digoxin or drugs that may cause hypomagnesaemia (e.g., diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment. Fracture: Proton pump inhibitors, especially if used in high doses and over long durations (>1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognised risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10-40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of vitamin D and calcium. Clostridium Difficile Diarrhea: Published observational studies suggest that PPI therapy may be associated with an increased risk of Clostridium difficile associated diarrhea, especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated. Vitamin B12 Deficiency: Daily treatment with any acid-suppressing medications over a long period of time (e.g., longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy have been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed. Interference with laboratory tests: Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient(s) are due to have a test on Chromogranin A level, Oprazol treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see section Pharmacodynamic). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

Use In Pregnancy And Lactation:
Risk benefits should be considered although there is no evidence of foetal toxicity or teratogenic effect.

Drug Interactions:
As omeprazole is metabolised by the hepatic cytochroma P450 enzyme system, it may increase the plasma concentrations of diazepam, phenytoin and warfarin, due to the decreased rate of metabolism by the liver. Due to increased pH of stomach by omeprazole reduction of absorption of ampicillin esters, iron salts and ketoconazole may be noted.

Contraindications:
It is contraindicated in patients hypersensitive to omeprazole.

Dosage and Administration:
Benign gastric ulcers: 20 - 40mg daily for 8 weeks.
Duodenal ulcers: 20 - 40mg daily for 4 weeks, Long term therapy for patients with a history of recurrent duodenal ulcer is recommended at a dosage of 20mg omeprazole once daily, up to one year.
Healing of reflux oesophagitis: 20 - 40mg once daily for 4 - 8 weeks.
Zollinger-Ellison syndrome: 60mg once daily. Doses in the range of 20 - 120mg daily may be used. Doses above 80mg should be divided and given twice daily. The dosage should be adjusted individually and treatment continued as long as is clinically indicated.

Note:
Impaired renal function: Dosage adjustment is not required in patients with impaired renal function.
Impaired hepatic function: As bioavailability and plasma half-life of omeprazole are increased in patients with impaired hepatic function, a daily dose of 20mg may be sufficient.
Elderly: Dose adjustments is not required.

Treatment of overdosage:
Symptoms: confusion, drowsiness, blurred vision, tachycardia, nausea, diaphoresis, flushing, headache and dry mouth. Treatment: symptomatic and supportive.

Storage conditions	:	Keep out of reach of children. Store in cool, dry place, below 30°C. Protect from light.
Presentation	:	5 x 10's/blister
Shelflife	:	3 years
Product Registration holder & Manufacturer	:	Prime Pharmaceutical Sdn. Bhd. 1505, Lorong Perusahaan Utama 1, Taman Perindustrian Bukit Tengah, 14000 Bukit Mertajam, Pulau Pinang.

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